

ECLIPS Internal Workshop 2

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Preliminary Results

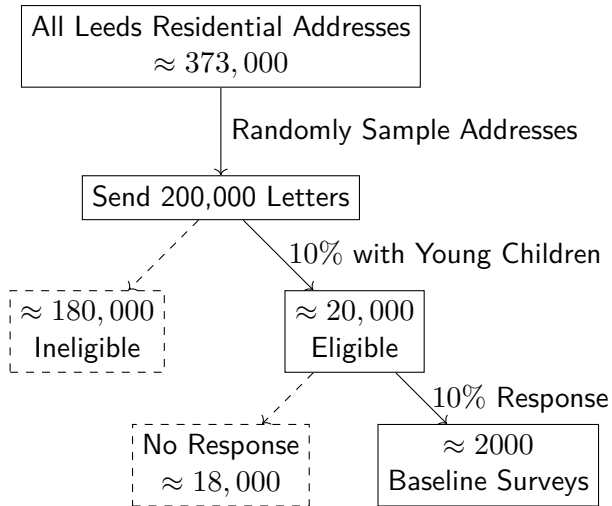
Phase I: Recruiting / Study Participation Decision

Make a simplified flowchart: Frank/Ludovica birds-eye view of Phase I and Phase II. Then “zoom in” on specific parts and fill in details. Could be more like Angela’s flowchart initial boxes. E.g. addresses, letters, randomization, re-balancing, dust kits, feedback

1. Sampling design
 2. Recruitment / Intake (don't mention community pathway in this talk!)
 3. Kit allocation & Randomization & Exposure Survey
- ▶ *These are the things we will analyze today; later we may look at dust / feedback surveys*
 - ▶ Start with super simple 3 blocks flowchart
 - ▶ As we progress through the talk give some more detail; possibly show more flowcharts with specifics (or not)

Phase I: Recruiting / Study Participation Decision

Update this by removing ineligible / eligible and putting actual responses



5 GBP gift card for sign-up and baseline survey completion

Research Question: who responds and why?

- ▶ Various kinds of responses: surveys, returning kits, experience surveys etc.
- ▶ Design of recruiting materials: RCT
- ▶ Design of the kits: RCT
- ▶ Demographic characteristics (don't observe at the individual level but do observe at LSOA); do they look like the population of Leeds overall?
- ▶ Can look at this for each kind of response

Sampling Design

- ▶ What is our sampling frame? What kinds of exclusion criteria? (removed student residences, hotels, commercial blah blah)
- ▶ What external data sources did we use? (Census, EPCs, Leeds Council Tax, Postal Codes)
- ▶ How / why did we stratify and what did we stratify on? Estimate share eligible (child 1 to 6; how we approximated it)
- ▶ Also stratify by housing age and LSOA (say how many LSOAs)

Map of where we sent letters

- ▶ Map of where we sent letters for both rounds; possibly next to or alongside a plot of the estimated share eligible across LSOAs.

Describe the treatments

- ▶ This slide gives the text and the motivation
- ▶ One treatment is only for old houses
- ▶ Definition of old house

Show the different recruiting letters

- ▶ This slide shows images (or put on prev slide if it fits)

Re-balancing / Kit Allocation

- ▶ Recruiting phase is to find people who will get kits
- ▶ Intentionally recruit more people than kits so we can re-balance
- ▶ Why re-balance? We expect that response related to demographic characteristics and lead exposure is too but in opposite directions
- ▶ How we re-balanced and how we assigned kits: another RCT here with two kit types.
- ▶ Could just say what the totals were for each round without explaining the results in detail.
- ▶ (For the moment don't talk about the two round aspect; that comes next)

Map of where we sent kits?

- ▶ Haven't looked at this yet; might look quite different because of re-balancing

Adaptive Design

- ▶ Everything above is correct but doesn't explain how we broke Phase I (Intake) into two rounds
- ▶ 150k total letters of which 30k in round 1 and adaptive round 2
- ▶ Explain how / why we did this adaptively and our pre-registered rule
- ▶ Explain how the re-balancing interacted with the two-stage process

Description of outcomes

- ▶ Define “response” as response within 45 days of sending letter
- ▶ Define “usable” response as: GP, valid other stuff... (check this)
- ▶ How do we define “returning a blood kit”? We don’t have a deadline as in the intake survey, but at some point we stop the study!

Basic Results: Intake Survey

- ▶ How many usable responses did we get? Overall / (stable by round) / by treatment / by age of house
- ▶ This is not a set of regressions (per se) it's just raw shares by cell (treatment / kind of house / histogram or map by LSOA)
- ▶ Could compare against our protocol where we predicted response rates based on similar studies; could talk about the per participant cost of recruiting in this way (need the total letter cost)

Heterogeneity: Intake Survey

- ▶ Frank did a bunch of crazy stuff!
- ▶ Various regressions; can we show them graphically? (Probably yes: use a map)
- ▶ Can we show regressions? (Yes we can!)
- ▶ Index of multiple deprivation is particularly interesting here but it's actually *not* a census characteristic. Should point out where we got it from.

Non-usable Responses: Intake Survey

- ▶ Are there any treatment effects for *clicking on the survey* or *starting the survey* even if you don't finish?
- ▶ Does this vary with anything we can observe? Treatment / housing age etc?
- ▶ This will require Frank to check with Caner; something is weird about the updated anonymized data: doesn't seem to have removed any of the "testing" rows

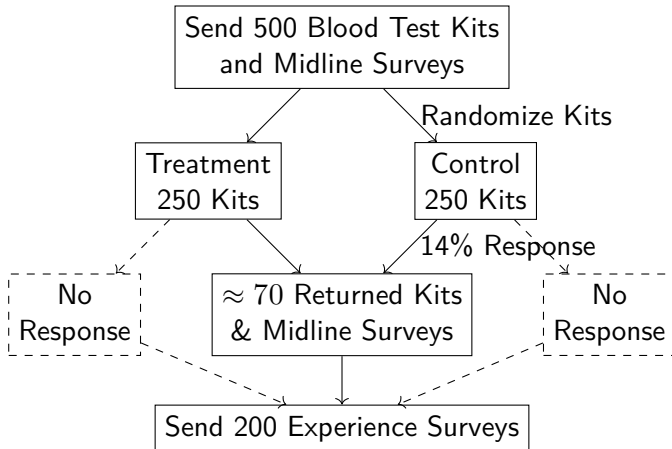
Intake Survey Descriptive Statistics

- ▶ Perhaps start with some basic descriptives from the survey
- ▶ Would be nice to say something like: based on census data (and where we sent letters) we would expect: the overall sample / this MSOA to have such and such shares of different kinds of people
- ▶ In the sample we actually had: give characteristics.
- ▶ Intake survey: did people know about lead already? Are we getting people who already knew all about lead or people who didn't know much about lead in advance?

Phase II: Collecting Blood Samples

Update numbers in this flowchart; can give current estimate of response rates although still accepting round 2 kits

Baseline survey & census: choose probs of sending each household a kit (2000 to 500)



Who returns a blood sample?

- ▶ Needs to be put into the repo in the anonymized form and merged.
- ▶ Talk to Andrea about this today!
- ▶ Consider adding some very preliminary results about who sends back a kit (valid kit?)
- ▶ Look at this by “golden envelope” treatment
- ▶ Could also correlate with demographics / knowledge about lead etc. These need to be re-weighted so they're slightly trickier. Use the re-balancing weights.

Who returns the exposure survey?

- ▶ Ask Andrea about this; she has sent the vouchers for this so we can probably modify her existing code to do this; should already be anonymized and ready-to-use?
- ▶ Can correlate this with the same things as returning a blood sample: kit type, demographics, knowledge about lead, etc.

Feedback survey

- ▶ It was opened a few days ago we think? Ask Andrea
- ▶ Perhaps we could present some very preliminary results?

Categorizing letters

- ▶ RA is still going through all the letters that were returned
- ▶ Hasn't finished yet
- ▶ We could add a slide here explaining what we plan to do with that information when we get it.
- ▶ E.g. we could try to correct the response rate estimates and we could check to make sure that this wasn't related to LSOA characteristics or we might find out that it is!